

Services Provided for the Diagnosis and Treatment of Diabetic Sensory Neuropathy with Loss of Protective Sensation (aka Diabetic Peripheral Neuropathy)

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Tracking Information

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Manual Section Number

70.2.1

Manual Section Title

Services Provided for the Diagnosis and Treatment of Diabetic Sensory Neuropathy with Loss of Protective Sensation (aka Diabetic Peripheral Neuropathy)

Version Number

1

Effective Date of this Version

07/01/2002

Implementation Date

07/01/2002

Description Information

Benefit Category

Physicians' Services

Please Note: This may not be an exhaustive list of all applicable Medicare benefit categories for this item or service.

Item/Service Description

Presently, peripheral neuropathy, or diabetic sensory neuropathy, is the most common factor leading to amputation in people with diabetes. In diabetes, sensory neuropathy is an anatomically diffuse process primarily affecting sensory and autonomic fibers; however, distal motor findings may be present in advanced cases. Long nerves are affected first, with symptoms typically beginning insidiously in the toes and then advancing proximally. This

leads to loss of protective sensation (LOPS), whereby a person is unable to feel minor trauma from mechanical, thermal, or chemical sources. When foot lesions are present, the reduction in autonomic nerve functions may also inhibit wound healing.

Indications and Limitations of Coverage

Diabetic sensory neuropathy with LOPS is a localized illness of the feet and falls within the regulation's exception to the general exclusionary rule (see 42 CFR §411.15(l)(1)(i)). Foot exams for people with diabetic sensory neuropathy with LOPS are reasonable and necessary to allow for early intervention in serious complications that typically afflict diabetics with the disease.

Effective for services furnished on or after July 1, 2002, Medicare covers, as a physician service, an evaluation (examination and treatment) of the feet no more often than every six months for individuals with a documented diagnosis of diabetic sensory neuropathy and LOPS, as long as the beneficiary has not seen a foot care specialist for some other reason in the interim. LOPS shall be diagnosed through sensory testing with the 5.07 monofilament using established guidelines, such as those developed by the National Institute of Diabetes and Digestive and Kidney Diseases guidelines. Five sites should be tested on the plantar surface of each foot, according to the National Institute of Diabetes and Digestive and Kidney Diseases guidelines. The areas must be tested randomly since the loss of protective sensation may be patchy in distribution, and the patient may get clues if the test is done rhythmically. Heavily callused areas should be avoided. As suggested by the American Podiatric Medicine Association, an absence of sensation at two or more sites out of 5 tested on either foot when tested with the 5.07 Semmes-Weinstein monofilament must be present and documented to diagnose peripheral neuropathy with loss of protective sensation.

The examination includes:

1. A patient history, and
2. A physical examination that must consist of at least the following elements:
 - Visual inspection of forefoot and hindfoot (including toe web spaces);
 - Evaluation of protective sensation;
 - Evaluation of foot structure and biomechanics;
 - Evaluation of vascular status and skin integrity;
 - Evaluation of the need for special footwear; and
3. Patient education.

A. Treatment includes, but is not limited to:

- Local care of superficial wounds;
- Debridement of corns and calluses; and
- Trimming and debridement of nails.

Claims Processing Instructions

TN AB-02-158 (Program Memorandum Intermediaries/Carriers) [↗](#)

[TN AB-02-096 \(Program Memorandum Intermediaries/Carriers\)](#) 

[TN AB-02-109 \(Program Memorandum Intermediaries/Carriers\)](#) 

[TN A-02-039 \(Program Memorandum Intermediaries\)](#) 

[TN AB-02-042 \(Program Memorandum Intermediaries/Carriers\)](#) 

[TN 498 \(Medicare Claims Processing Manual\)](#) 

Transmittal Information

Transmittal Number

153

Coverage Transmittal Link

<https://www.cms.gov/Regulations-and-Guidance/Guidance/Transmittals/Downloads/R153CIM.pdf> 

Revision History

05/2014 - CMS translated the information for this policy from ICD-9-CM/PCS to ICD-10-CM/PCS according to HIPAA standard medical data code set requirements and updated any necessary and related coding infrastructure. These updates do not expand, restrict, or alter existing coverage policy. Implementation date: 10/06/2014 Effective date: 10/1/2015. ([TN 1388](#) ) ([TN 1388](#) ) (CR 8691)

09/2012 - CMS translated the information for this policy from ICD-9-CM/PCS to ICD-10-CM/PCS according to HIPAA standard medical data code set requirements and updated any necessary and related coding infrastructure. These updates do not expand, restrict, or alter existing coverage policy. Implementation date: 01/07/2013 Effective date: 10/1/2015. ([TN 1122](#) ) ([TN 1122](#) ) (CR 7818)

04/2002 - Provided coverage as a physician service under section 1861(s)(1) of the Act. Effective and implementation dates 07/01/2002. ([TN 153](#) ) (CR 2060)

National Coverage Analyses (NCAs)

This NCD has been or is currently being reviewed under the National Coverage Determination process. The following are existing associations with NCAs, from the National Coverage Analyses database.

- [Original Consideration for Diabetic Peripheral Neuropathy with Loss of Protective Sensation \(LOPS\) \(CAG-00059N\)](#) 

Additional Information

Other Versions

Title	Version	Effective Between	
Services Provided for the Diagnosis and Treatment of Diabetic Sensory Neuropathy with Loss of Protective Sensation (aka Diabetic Peripheral Neuropathy)	1	07/01/2002 - N/A	You are here